# Community Assessment, Prevention, and Population Reasoning

NURSING · nursing-foundations · BoardPrep lesson v1

## Official examination scope

PRC Resolution No. 10 s. 2025 and the August 2026 NLE program define examination scope, not topic weights. This lesson teaches a bounded community assessment and prevention foundation from the references below. Open RN and U.S. professional materials support general educational principles only; apply Philippine law, scope of practice, current clinical orders, referral pathways, and facility policy. This study material is not a clinical directive or proof of complete NP coverage.

## Learning objectives

- Assess a community concern with the affected people and relevant service partners before
- selecting an intervention.
- Distinguish population prevention levels from condition-specific uses of prevention
- terminology and keep individualized care plans with the treating team.
- Calculate and interpret a simple event proportion using the correct at-risk denominator.
- Recognize how access barriers and new safety changes affect assessment and referral
- priorities.

## 1. Start with the community’s problem definition

Community assessment is a structured inquiry into health patterns, resources, barriers, and priorities with the people affected. Begin with a defined population and question: who is missing care, what changed, where, and when? Combine available records with resident and frontline-worker perspectives. Check whose experience is absent, whether access, language, transport, cost, safety, or service hours are relevant, and what community strengths already exist. A proposed program should fit local resources and be designed with, not merely delivered to, residents. This is a general method; Philippine barangay structures, referral routes, and program eligibility must be confirmed locally.

## Sources for this section

Section reference: 16.2 Community Health Concepts — Nursing Mental Health and Community Concepts 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingmhcc/chapter/16-2-community-health-concepts/
Section reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/
Section reference: Primary health care and community health guidance — World Health Organization — https://www.who.int/health-topics/primary-health-care

## 2. Separate prevention levels from a disease-specific plan

In the common health-promotion framework, primary prevention acts before disease, secondary prevention seeks earlier detection, and tertiary prevention limits consequences of established disease. The same word can be used more narrowly by a specialty guideline: “secondary stroke prevention” describes reducing recurrent stroke risk after a prior event. Do not mechanically relabel that care as screening or infer a drug, activity limit, laboratory target, or follow-up interval. Support adherence to the plan made by the treating team, clarify questions, reinforce warning-sign and follow-up instructions, and escalate new symptoms through the current pathway. The AHA/ASA sources are U.S. references and require currentness and Philippine-applicability review.

## Sources for this section

Section reference: 2.2 Health, Wellness, and Disease Prevention — Nursing Health Promotion — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/healthpromo/chapter/2-2-health-wellness-and-disease-prevention/
Section reference: 2021 Guideline for the Prevention of Stroke in Patients With Stroke and Transient Ischemic Attack — American Heart Association and American Stroke Association — https://www.ahajournals.org/doi/10.1161/STR.0000000000000375
Section reference: 2026 Guideline for the Early Management of Patients With Acute Ischemic Stroke — American Heart Association and American Stroke Association — https://www.ahajournals.org/doi/10.1161/STR.0000000000000513

## 3. Make the denominator visible

An attack proportion is a descriptive fraction for a defined population at risk during a specified event or interval: cases in that group divided by people at risk in that group, multiplied by 100. It is not automatically an incidence rate, a causal effect, or proof of where an outbreak began. Verify the case definition and time window, describe cases by person/place/time, and compare the observed pattern with an appropriate baseline before drawing conclusions. Reporting requirements and investigation authority are jurisdiction-specific; the CDC field manual is used here for general epidemiologic method, not Philippine reporting rules.

## Sources for this section

Section reference: Conducting a Field Investigation — U.S. Centers for Disease Control and Prevention — https://www.cdc.gov/field-epi-manual/php/chapters/field-investigation.html

## 4. Assess vulnerability without stereotyping

Vulnerability describes how exposure, resources, health, and structural barriers can combine to increase harm; it is not a fixed label or a reason to remove a person’s choices. Ask what the person can safely do, what support is available, what they prefer, and what barriers can be changed. A new change such as confusion is not ordinary aging by default: compare with baseline, assess for acute illness or injury and environmental hazards, and seek timely help under the local pathway. Protect dignity while addressing immediate risk, then involve the person in a feasible plan.

## Sources for this section

Section reference: 17.2 Vulnerable Populations — Nursing Mental Health and Community Concepts 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingmhcc/chapter/17-2-vulnerable-populations/
Section reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/

## Map a missed-visit barrier

Question: Older adults with a chronic condition miss follow-up because transport is unreliable. What should the nurse learn first?
1. Define which residents and visits are affected and over what period.
2. Ask residents and local workers how transport, cost, route, timing, and available services shape access.
3. Use the findings to co-design a feasible outreach or referral response, then review whether access improved.
Answer: The first step is shared assessment of the barrier, not an education campaign chosen before the cause is known.
Check: The plan responds to evidence from the affected community and does not assume private transport is available.

## Use prevention language carefully

Question: A person asks about preventing another stroke after discharge.
1. Recognize that this is a condition-specific recurrent-risk context, not a generic screening question.
2. Reinforce the individualized plan, adherence, follow-up, and warning-sign instructions already set by the treating team.
3. Refer medication, activity, or risk-target questions to the authorized clinician and current local guidance.
Answer: Support the existing care plan and clarify or escalate questions; do not prescribe or replace the plan.
Check: No supplement, medication, target, or activity restriction is invented.

## Calculate an event proportion

Question: There are 18 cases among 120 residents at risk during one defined week.
1. Check that numerator and denominator refer to the same population and interval.
2. Compute 18 ÷ 120 = 0.15.
3. Convert 0.15 × 100 = 15%.
Answer: The attack proportion is 15%.
Check: The result is a proportion for the stated group and interval; it does not establish cause.

## Respond to a new safety change

Question: An older adult who lives alone is suddenly confused and has an unsafe kitchen.
1. Compare the change with the person’s usual cognition and ask when it began.
2. Assess immediate hazards, possible illness or injury, and available support without assuming dementia.
3. Use the local urgent evaluation/referral pathway and include the person in decisions as able.
Answer: Assess acute change and immediate safety, then escalate; do not label it normal aging.
Check: The response protects choice while treating new confusion as a potentially urgent cue.

## Common misconceptions

- Community assessment is not a one-way health lecture; it begins by learning the affected
- group’s priorities and barriers.
- Primary, secondary, and tertiary prevention labels are not interchangeable with every
- specialty use of “secondary prevention.”
- A percentage does not establish incidence, causation, or outbreak source without a defined
- population and interval.
- Older age does not make new confusion expected; confirm baseline and assess for acute change.
- A vulnerable-population label does not replace an individual assessment or justify removing
- autonomy.

## Practice patterns

- For access barriers, identify the affected population and ask before choosing a service
- response.
- For post-event prevention, reinforce the treating team’s individualized plan and avoid
- creating a regimen.
- For a proportion, state numerator, denominator, and time window before calculating.
- For a new safety change, compare with baseline and escalate according to current local
- pathways.

## Official scope references

Official scope: Enhanced Tables of Specifications for the Subjects in the Nurses Licensure Examination — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/2025-10%20%20published%20Enhanced%20TOS%20nuse.pdf
Official scope: Nursing Licensure Examination Program, August 2026 — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/exam%20progam%20aug%202026%20nle%20(nursing)%20signed.pdf

## Instructional references

Instructional reference: 16.2 Community Health Concepts — Nursing Mental Health and Community Concepts 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingmhcc/chapter/16-2-community-health-concepts/
Instructional reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/
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Instructional reference: 2026 Guideline for the Early Management of Patients With Acute Ischemic Stroke — American Heart Association and American Stroke Association — https://www.ahajournals.org/doi/10.1161/STR.0000000000000513
Instructional reference: Conducting a Field Investigation — U.S. Centers for Disease Control and Prevention — https://www.cdc.gov/field-epi-manual/php/chapters/field-investigation.html
Instructional reference: 17.2 Vulnerable Populations — Nursing Mental Health and Community Concepts 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingmhcc/chapter/17-2-vulnerable-populations/

BoardPrep lesson material is original instruction. It is not an official examination document.
No claim about question frequency or examination weighting is made.